Document Control

Title

Protocol for the Management of Traumatic Neck Pain in Minor Injury Units

Author	**Author's job title** Professional Lead, Minor Injuries Unit
Directorate Emergency Services, Logistics and Resilience	**Department** Emergency Department

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1.1	Apr 2016	Revision	Reviewed and amended Karen Watts / Fionn Bellis
2.0	Nov 2017	Final	Approved at the Drugs and Therapeutics Committee.

Main Contact Emergency Department North Devon District Hospital Raleigh Park Barnstaple, EX31 4JB	**Tel: Direct Dial** – **Tel: Internal** – **Email**:
Lead Director Medical Director	
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Consulted with the following stakeholders: (list all)

- ED Consultants and Associate Specialists
- MIU Leads

Approval and Review Process

- Lead Clinician for Emergency Department

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1. Table of Contents

Document Control **1**
2. Purpose **3**
3. Presenting Symptoms **3**
3.1 History 3
3.2 Clinical Examination 3
4. Treatment Pathway **4**
5. Discharge Pathway **5**
5.1 DOCUMENTATION TO BE COMPLETED 5
5.2 BEFORE DISCHARGE ENSURE 5
6. References **6**
APPENDIX A – Essential Documentation for All Patients Attending Unit or Centre **7**
APPENDIX B – Essential Documentation for All Patients Attending Unit or Centre **8**
APPENDIX C – Training Competency Form **9**

2. Purpose

This Protocol is for the use by staff employed by Northern Devon Healthcare Trust who have achieved the agreed clinical competencies to work under this protocol.

3. Presenting Symptoms

Neck pain with definitive mechanism of injury

3.1 History

Refer to protocol for History taking and Clinical Documentation Protocol.

- Accurate assessment of a patient suffering neck pain requires a detailed history of the mechanism of injury to be recorded
- Document carefully the presence or absence of any associated neurological signs
- Record any delay in onset of pain and any associated symptoms
- Document how they arrived to the clinical environment and if they were ambulatory at any stage
- Document any self-care the patient has administered

3.2 Clinical Examination

Blood pressure, pulse, respiratory rate, oxygen saturations should be recorded.

1. Not every patient will require full spinal immobilisation and transfer by ambulance, however patient safety is paramount. If the nurse is in any doubt then the patient must be immobilised and transferred by ambulance.

2. Patients with neck pain or stiffness following injury, and any one of the following risk factors, should be immobilised and transferred to the nearest DGH:

- GCS less than 15 at the time of assessment
- Age greater than or equal to 65 years
- Dangerous mechanism of injury (see box A)
- Paraesthesia in extremities, upper / lower limbs
- Focal peripheral neurological deficit

Dangerous mechanism of injury

- Neck pain or tenderness
- Fall from greater than or equal to 1 metre/5 stairs
- Axial load
- To head, e.g. Diving

- High speed motor vehicle collision (>100Km/Hr.), roll over, ejection
- Motorised recreational vehicles
- Bicycle collision

Patients not immobilised under assessment point 2, should be assessed for the presence or absence of low risk factors that allow for the safe assessment of range of movement.

Any low risk factors that allows safe assessment of range of motion?

Simple rear end motor vehicle collision (†)
Or
Sitting position in department
Or
Ambulatory at any time since injury
Or
Delayed onset of neck pain (‡)
Or
Absence of midline cervical spine tenderness

Simple rear-ends motor vehicle collision Excludes:

- Pushed onto oncoming traffic
- Hit by Bus/ Large Truck
- Rollover
- Hit by high speed vehicle

Delayed

- Not immediate onset of neck pain

Patients not immobilised under assessment point 2, who have one or more of the low risk factors that allow safe assessment of range of movement, should be asked to actively rotate their neck 45 degrees to the left and right. If the patient is unable to complete this movement they should be immobilised and transferred.

If in any doubt contact the Emergency Department (ED) for advice

4. Treatment Pathway

If any risk factors found or concerned about presentation then the patient is to have a stiff neck collar fitted and is to be immobilised whilst awaiting transfer via 999 ambulance to the Emergency Department

Offer appropriate analgesia as per PGDs

Patients found not to require immobilisation under management points 2 and 4, may be discharged with advice on self-care of acute neck sprain. Advise the patient on:

1. The need for regular analgesia

2. Regular, gentle neck exercises

3. To seek medical advice if:

- The pain continues to worsen
- The pain persists beyond 4 weeks
- Any numbness, weakness or pins and needles develop

5. Discharge Pathway

Assess and document pain score prior to discharge

Ensure patient is issued with appropriate advice sheet (if available) and that patient understands the need to return if symptoms change or worsens.

Discuss home analgesia with patient, parent or carer and advise OTC medication or
administer TTO medication as per PGD.

5.1 DOCUMENTATION TO BE COMPLETED

- Clinical treatment record as per Documentation & record keeping policies. Copy of clinical treatment record to General Practitioner; to be sent to surgery as per Record keeping policy.
- For patients being transferred to secondary care, ensure a copy of the clinical treatment record is sent with patient. A copy will also be sent to surgery in normal manner.

5.2 BEFORE DISCHARGE ENSURE

- Those patients who have been referred for further acute intervention has appropriate transport to meet their needs, all relevant treatment has been prescribed and administered and correct information and documentation is given to the patient.
- The patient understands that if condition deteriorates or they have further concerns they should seek further advice.
- The patient demonstrates understanding of advice given during consultation.
- The patient demonstrates an understanding of how to manage subsequent problems.

6. References

Purcell D. (2010) Minor Injuries: a Clinical Guide (2nd Edition) Edinburgh Churchill Livingston

NICE Guidelines (CG176) January 2014

Emergency Department Guidelines (2012) NDHCT

Protocol for History Taking and Clinical Documentation (2015) NDHCT

Consent Policy (2019) NDHCT

Safeguarding Children Policy (2018) NDHCT

Patient Group Direction Policy (2016) NDHCT

Medicines Policy (2018) NDHCT

Patients at Risk of Deterioration Policy (2019) NDHCT

APPENDIX A – Essential Documentation for All Patients Attending Unit or Centre

Adults Consent
Gain consent to be seen by a nurse practitioner
Gain consent for treatment and sharing information and document.

Clinical Presentation
If unwell assess for:

- Airway
- Breathing
- Circulation
- Disability
- Exposure

Document a full set of observations including neurological observations including Glasgow coma score if applicable.

Record NEWS or PEWS to determine referral to GP or secondary care if appropriate.

Document pain score using numeric rating scale. For cognitively impaired patients document any signs of pain (e.g. grimaces or distress).

Safeguarding

- Ask the domestic abuse question, 'do you feel safe at home?'
- Assess for mental capacity and if the person is a vulnerable adult.
- Assess for learning disability and whether the person has a hospital passport in place.
- Assess falls risk. Complete falls referral if applicable.

APPENDIX B – Essential Documentation for All Patients Attending Unit or Centre

Child and Young Persons under 18 Years Old Consent
Gain consent to be seen by a nurse practitioner
Gain consent for treatment and sharing information
Assess and document competence according to Fraser guideline if applicable.
Document name of person(s) accompanying patient

Clinical Presentation

If unwell assess for:

- Airway
- Breathing
- Circulation
- Disability
- Exposure

Record PEWS: if any one parameter is triggered transfer to secondary care or seek advice from medical practitioner.

Use guideline Traffic Light System (NICE) 2013 if applicable.
Use guideline Feverish Illness (NICE) 2013 if applicable.
Document pain score using FLACC, Wong Baker Faces or numeric rating scale.

Safeguarding

- Complete safeguarding children's question (NICE 2013)
- Any injury or bruise on a non-mobile infant or child: follow safeguarding Children Policy (2018). These children must be reviewed by a Consultant in Emergency Medicine or a Consultant Paediatrician and a MASH (multi agency safeguarding hub) referral must be made.

DOCUMENT ALL FINDINGS IN THE CLINICAL TREATMENT RECORD AND ACT ON THEM FOLLOWING NDHCT GUIDELINES.

APPENDIX C – Training Competency Form

Protocol for the Management of Traumatic Neck Pain

Protocol operational from September 2019 and expires end of September 2022

The registered health professional named below, being employees of Northern Devon Healthcare Trust based at ……. have received training and are competent to operate under this protocol

NAME *(please print)*	PROFESSIONAL TITLE	SIGNATURE	AUTHORISING MANAGER *(please print)*	MANAGER'S SIGNATURE	DATE

Keep original with the authorising manager and send a copy to: Karen Watts, Emergency Department, Northern Devon Healthcare Trust NHS, Raleigh Park, Barnstaple, Devon, EX31 4JB